

# Nurse AI OS™ — Media Packet

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nurse-ai-os.org · July 2026 · Media contact: [great.ai.nurses@gmail.com](mailto:great.ai.nurses@gmail.com)

*Ready-to-use language for three audiences — the general public, nursing organizations, and AI architects/technology press — plus fast facts, founder bio, accuracy notes, and assets. Everything in this packet may be quoted directly.*

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## Executive summary

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**Nurse AI OS™ is the commercial product brand for a governed, no-PHI AI learning and stewardship experience built for nurses by a 22-year ICU nurse.** It helps nursing students, bedside nurses, managers, educators, and nurse entrepreneurs build practical AI workflows around learning, organization, administration, and professional development without handing over judgment. Official workflows use EDENA risk tiers, human-review gates, and a strict rule against placing raw patient information in prompts, memory, telemetry, logs, or model context. The product can be used browser-first or with Hermes Agent as an external open-source runtime. Nurse AI OS is not the name of the underlying runtime, model, or general technical substrate; it is the protected identity of the nurse-centered commercial product and experience.

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## Fast facts

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- **What:** the commercial brand for a governed, no-PHI AI learning and stewardship product purpose-built for nurses; available browser-first or with Hermes Agent as an external runtime.
- **Who it serves:** six English post-setup lanes on one governance spine — four governed self-install Hermes build kits for Nursing Student and Nursing Assistant/FUTURE, Staff Nurse/SHIFT, Nurse Leader/LEAD, and Nurse Educator and Instructional Designer/TEACH; one Complete Edition for USA-only Nurse Practitioner/WINGS; and one review-first Nurse-Connected Ally overlay — plus advanced builder pathways. Downloading any lane does not install, activate, operationalize, or grant institutional authority.
- **Cost:** free for nurses and nursing students during the founding year.
- **The hard boundary:** no patient information in the product experience — “if it could identify a patient, leave it out.” No patient-specific clinical decisions. Official distributions are workforce, administrative, and educational infrastructure, not clinical decision support.
- **The safety model:** EDENA risk tiers (🟢 proceed / 🟡 human approves first / 🟠 deferred until governed review / 🔴 stop) — tiers measure risk, not privilege. Doctrine: *Agents propose. Humans judge. Nurses steward.*

- **The engineering:** signed releases (pinned key fingerprint → signed manifest → per-file checksums, fail-closed), human-staged approval for AI-authored skills, refuse-at-intake memory rules, and an anti-anchoring “judgment first” pattern.
  - **The evidence discipline:** research claims distinguish verified evidence, source-reported claims, and design judgment; corrections are published when evidence changes.
  - **Founder:** Robert Domondon, RN, CCRN, CSC, CMC — 22 years in the ICU, 13 years in healthcare management, physician training (Philippines), former US Naval Hospital administrator.
  - **Ownership and licensing:** copyright and the Nurse AI OS trademark are held by Robert Domondon personally pending later written entity assignment. The trademark application is pending; no federal-registration claim is made. Original source code is Apache 2.0, documentation is CC BY 4.0, and trademark rights remain separate.
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## For the general public

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**The 50-word description.** Nurse AI OS™ is a governed, no-PHI AI learning and stewardship product for nurses. It helps with studying, certifications, organization, careers, and non-clinical administration while keeping consequential decisions behind human review. It can start in a browser and later use Hermes Agent as an external runtime, with nurse-led governance throughout.

**The 150-word description.** Nurses are among the most trusted professionals in the world, and many are already exploring AI for everyday help. Nurse AI OS™, built by a 22-year intensive-care nurse, gives them a governed place to learn and build practical workflows without treating AI output as judgment. Tasks are classified by risk. Consequential work waits for human review. Raw patient information does not belong in prompts, memory, telemetry, logs, or model context. The product focuses instead on the life and work around care: exam preparation, certification planning, after-shift reflection, non-clinical administrative drafts, professional portfolios, and responsible nurse entrepreneurship. Users can begin browser-first and later choose Hermes Agent as an external runtime. Nurse AI OS is the commercial product brand—not the underlying model or runtime—and it is free for nurses and nursing students during its founding year. The motto tells the story from the start: *Agents propose. Humans judge. Nurses steward.*

### Three key messages.

1. **Nurses are already exploring AI — this teaches governed use.** The choice is not simply AI or no AI; it is whether use has boundaries, review, and accountable stewardship.
2. **Patient privacy is a hard participation rule.** Users must not place raw patient information in the product experience, prompts, memory, telemetry, logs, or model context.
3. **The human stays in charge.** Every consequential action waits for the nurse’s approval. The agent proposes; the nurse decides.

**Suggested angles.** The ICU nurse who spent 22 years at the bedside and built an AI product on nursing’s checklist culture · What every profession can learn from how nurses are governing AI · “If it could identify a patient, leave it

out”: the nurse-led product with a hard no-PHI boundary.

**Pull quote.** *“Nurses shouldn’t just use AI — they should govern it.”* — Robert Domondon, founder

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## For nursing organizations

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**Boilerplate paragraph (quote freely).** Nurse AI OS™ is a nurse-led commercial product for governed, no-PHI AI learning and stewardship. Its design is informed by published guidance from the American Nurses Association, NCSBN, and the American Academy of Nursing: AI should augment rather than replace nursing judgment; accountable humans remain in the loop; and capabilities require validation before institutional use. Official workflows use EDENA risk tiers and human-review gates. Raw patient information is prohibited from prompts, memory, telemetry, logs, and model context. Academic-integrity workflows emphasize disclosure, process evidence, and educator review rather than automated accusation. Nurse leaders receive practical governance templates, but neither the product nor those templates confer institutional approval, compliance status, clinical readiness, or certification. Nurse AI OS is free for nurses and nursing students during its founding year.

### Talking points.

- **It translates existing guidance into practice rather than claiming institutional authority.** A guidance-to-structure mapping is published in the Architecture Report with citations and explicit claim boundaries.
- **It builds the AI-governance workforce the guidance assumes.** Every user practices daily inside risk tiers, review gates, escalation rules, and decision ledgers — the instincts organizations need as clinical AI arrives. Surveyed nursing programs report a large AI-literacy gap; the Student module is aimed squarely at it.
- **Academic integrity is handled the way the evidence says to.** Detection tools have documented false-positive problems; the Study OS builds integrity on disclosure templates, process logs, and educator review-and-interview workflows — never automated accusation.
- **Nurse review is structural.** Shared artifacts are designed for nurse review against published checklists and human gates. Publication is not endorsement, certification, compliance approval, or clinical validation.
- **What it is not (say this plainly):** not clinical decision support, not connected to the EHR, not a substitute for institutional governance, not a certification of competence, and not evidence of improved patient outcomes.

**The collaboration invitation.** Nursing organizations, schools, and unions interested in pilots, workshops (a 90-minute unit-ready session ships in the kit), or governance collaboration can reach the project at [great.ai.nurses@gmail.com](mailto:great.ai.nurses@gmail.com).

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## For AI architects and technology press

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**Boilerplate paragraph (quote freely).** Nurse AI OS™ is a commercial nurse-centered product that can use Hermes Agent as an external open-source runtime. It does not rename, absorb, or claim ownership of that runtime or of a general AI substrate. The product adds nurse-facing governance files, EDENA risk classification, human-review patterns, release verification, and no-PHI boundaries around supported workflows. Where controls are mechanically enforced, the documentation says so; where behavior still depends on model instructions or human review, that limitation is stated. Hermes Agent is not vendored in the Nurse AI OS repository. OpenClaw is referenced for interoperability and architectural research, not distributed as a parallel runtime. Original Nurse AI OS source code is Apache 2.0, documentation is CC BY 4.0, and upstream components retain their own licenses and notices.

### Talking points.

- **Gates, not vibes.** The design treats standing instructions as necessary but insufficient for redline enforcement. Mechanically enforced points (staged skill writes, command approval, signed-release refusal checks) are distinguished — explicitly and in public documentation — from model-followed patterns like the Yellow-tier self-audit footer, which is audited at the human gate; a delivery-gating output verifier is the named next hardening step. The project corrected its own documentation when a reviewer flagged the distinction. That correction habit is the story.
- **The agent can become part of its own supply chain.** When a configured runtime proposes or writes tools, those artifacts require provenance review, scanning, and human approval before promotion.
- **Anti-anchoring as skill structure.** For anything decision-adjacent, the human's assessment is elicited *before* AI material is shown — automation-bias mitigation encoded as interaction order, not advice.
- **A worked example of domain-informed governance.** The contribution is a practical nurse-centered product that translates published professional expectations into everyday risk classification, review, and escalation habits without claiming regulator or institution status.
- **Read the receipts:** the Architecture Report (HTML + PDF at [nurse-ai-os.org/architecture-report.html](https://nurse-ai-os.org/architecture-report.html)) carries the guidance-to-structure mapping and summarized citations; the naio-os standards crosswalk serves technical reviewers.

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## Founder

**Short bio (50 words).** Robert Domondon, RN, CCRN, CSC, CMC, is a critical-care nurse with 22 years at the ICU bedside, 13 years in healthcare management, physician training in the Philippines, and former administration of a US Naval-base hospital. He is the founder of Nurse AI OS, the NAIO Institute, and the Nurse Intelligence Network.

**Longer bio (120 words).** Robert Domondon's career runs the full span of healthcare: physician training and practice in the Philippines, hospital administration on a former US naval base, thirteen years in healthcare

management, AI-adoption consulting — and, by deliberate choice, more than two decades as a critical-care nurse in the United States. That combination is the design brief for Nurse AI OS: built by someone who has signed the budget, written the policy, and held the hand of the patient the policy was written for. His governing doctrine — “governance by design; clarity and values before velocity” — runs through the ecosystem he founded: the NAIIO Institute (governance education and standards work), the Nurse Intelligence Network (the professional community), EDENA (the ethics-and-risk layer), and Nurse AI OS itself.

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## The ecosystem at a glance

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- **Nurse AI OS™** — the commercial product brand and nurse-facing experience; never a synonym for the underlying runtime, model, or technical substrate.
  - **EDENA** — Ethical Design & Enablement for Nursing Augmentation: the risk-classification and gating layer every task runs under.
  - **NAIO Institute** — the initiative developing nurse-led AI governance education, standards work, and stewardship infrastructure; it does not confer legal, regulatory, or institutional approval.
  - **Florence-X** — the orchestration control plane for governed AI-agent workforces; humans retain final decision authority at every layer.
  - **Nurse Intelligence Network (NIN)** — the global professional community, summit, and podcast ([nurseintelligence.com](https://nurseintelligence.com)).
  - **Florence Media Network** — the content engine; every piece EDENA-classified and human-reviewed before release.
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## Accuracy notes — please describe us this way

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Getting these right matters more to this project than coverage does.

- **Do say:** “a governed, no-PHI AI learning and stewardship product for nurses” · “free for nurses and nursing students during the founding year” · “informed by published nursing guidance” · “can use Hermes Agent as an external open-source runtime” · “Nurse AI OS is the commercial product brand.”
- **Please don’t say:** “HIPAA-compliant” · “clinical AI,” “diagnoses,” or “decision support” · “certifies nurses” · “institutionally approved” · “improves patient outcomes” · “Nurse AI OS is the runtime, model, or substrate.”
- **The one-line boundary, quotable:** *“If it could identify a patient, leave it out. Agents propose. Humans judge. Nurses steward.”*

## Licensing and trademark

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Nurse AI OS™ is a trademark of Robert Domondon. The registration application is pending; no federal-registration claim is made. Robert also personally holds copyright in original project components. A later written assignment may transfer ownership to designated successor entities, but it changes the holder—not the published license, contributor, criticism, or permitted-use terms.

Original source code is Apache 2.0; documentation, playbooks, and curricula are CC BY 4.0. Contributors retain copyright, and project relicensing rights are limited to those published licenses. The contributor agreement contains no non-disparagement or critique-suppression term. Trademark governs identity—not criticism or open-source use. Full terms: [nurse-ai-os.org/licensing.html](https://nurse-ai-os.org/licensing.html) and the repository policy files.

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## Assets & links

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- **Site:** [nurse-ai-os.org](https://nurse-ai-os.org) · **Start Here:** [nurse-ai-os.org/start-here.html](https://nurse-ai-os.org/start-here.html) · **Pathways:** [nurse-ai-os.org/pathways.html](https://nurse-ai-os.org/pathways.html)
  - **Architecture Report** (for technical/executive readers): [nurse-ai-os.org/architecture-report.html](https://nurse-ai-os.org/architecture-report.html) (with PDF)
  - **Licensing and trademark:** [nurse-ai-os.org/licensing.html](https://nurse-ai-os.org/licensing.html)
  - **Care Intelligence White Paper** (June 2026): PDF
  - **Articles:** “Why Nurses Need an AI Operating System, Not Just an AI” · “Governance You Can Refute: The Evidence Behind Nurse AI OS” — both at [nurse-ai-os.org/nurse-station.html](https://nurse-ai-os.org/nurse-station.html)
  - **Community:** [nurseintelligence.com](https://nurseintelligence.com) · YouTube: [@NurseIntelligence](https://www.youtube.com/@NurseIntelligence) · Podcast: The Nurse Intelligence Podcast (Spotify)
  - **Founder photo:** available on request and at [nurse-ai-os.org/about.html](https://nurse-ai-os.org/about.html)
  - **Interviews, speaking, pilots, and governance inquiries:** [great.ai.nurses@gmail.com](mailto:great.ai.nurses@gmail.com)
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